

Patient 6**Report A**

6506938 24/05/2016 CT Angio (Head)

Clinical Indications for CT-Intracerebral haemorrhage; CTA to review for underlying structural abnormality

Comparison is made to the externally performed CT brain dated 20/05/2016.

A focus of high density is evident within the right occipital lobe measuring approximately 2.3cm x 1.5cm and is in keeping with an acute parenchymal haematoma. Perilesional low attenuation extending into the surrounding white matter reflects local vasogenic oedema. Appearances of the haematoma remain relatively stable compared to the previous CT brain.

The CTA study is suboptimal due to venous contamination.

There appears to be a slight increase in prominence of small vessels in the region of the haematoma, which may be due to local mass effect. No definite arteriovenous malformation/nidus is evident.

The dural venous sinuses and deep veins opacify normally.

An MRI brain is suggested to evaluate for an underlying cause.

Report B

6506987 24/05/2016 MR Head

Clinical history: New ICH with vasogenic oedema - MRI to identify underlying cause

Findings:

Comparison has been made with CT head 24/05/2016. There is a right occipital lobar haemorrhage with mild surrounding mass effect. There is a right P4 (cortical) PCA branch which skirts closely to the posterior mesial aspect of the haemorrhage, however no underlying arteriovenous malformation identified on MRI. No acute or established cortical infarct. No microhaemorrhage. The proximal branches at the circle of Willis are patent with no focal stenosis, occlusion or acute dissection. The imaged dural venous sinuses show normal signal void.

Comment: Right occipital lobar haemorrhage with no definite underlying AVM on MRI.

Small occult vascular malformation can not be excluded on MRI. Underlying lesion not excluded due to residual blood products. This should be followed to till resolution.